جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : انتصار جمال محمد محمد عطية
المحافظة : القاهره **الحي / المدينة :** الخليفه **العنوان :** *
رقم البطاقة : 28804120104524 **رقم التليفون :** 01055159019 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 12-04-1988 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: انتصار جمال محمد محمد عطية
National ID: 28804120104524

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

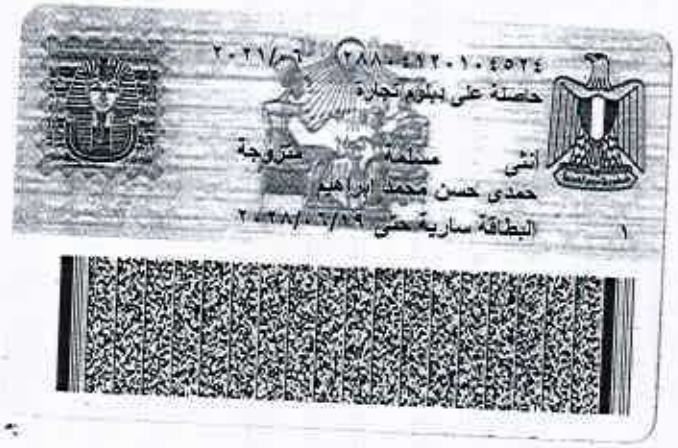
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
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Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسى لصحة المرأة
وزارة الصحة والسكان

MDT form

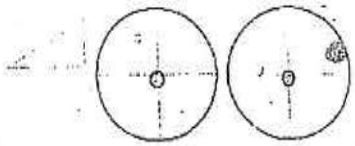
A-Patient ID:
1- Name : انتصار جمال محمد محمد

Sex: Female

3- Medical number: 83233183
Old Med : 3651-2024

2-ID number:

Patient Age : 37 Y



Family history	Family History -ve	Phone	Phone 01208463141	BMI	BMI	REASON OF MDT	REASON OF MDT NEW
marital status	Marital Status	NRS Score	NRS Score	GROUP	GROUP B	Date of MDT	Date of MDT 31/10/2
Co morbidity	Co Morbidity free	Menopausal status	Menopausal Status PREMENOPAUSAL	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :
lt breast irregular hypocholc lesion axilary taxil 24x20 mm
pathological axillary LN

Mets Workup :
abd pelvis us:free
CXR:free
bone scan:free

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC BX COMMENT: lt gli	ER COMMENT: +ve	PR COMMENT: +ve	Her 2 Neu COMMENT: -ve	SISH COMMENT :	18

T	N	M
CT2	1	

Sonomammography

PATIENT HISTORY
CESM THEN REMDT
CTCAP
LABS
ECHO EF 60%

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN
NACT ACX4 TAXOTERE X 4, ZOLADEX THEN SURGERY ACC TO CESM

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/ الاء جميل	RADIOLOGY	RADIOLOGY د/ رحاب البدري
SURGERY	SURGERY د/ ايهاب سامى	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

Name: انتصار جمال محمد	
MR. : 2969	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient underwent a left total mastectomy (lt mrm) on 12/7/2025 with a noted n8/19. In July 2025, a multidisciplinary team (mdt) discussion identified a treatment plan comprising Zoledek, PORT, and a new metastatic work-up. On 8/1/2026, the patient was started on arimidex, with additional documentation of ARAMIDEX CALCIUM 1*1*90, ONE ALPHA ZOLADEX 10.8 SC SM, and a breast lesion likely fibroadenoma on RT breast imaging. The plan included a BRIAD 3 for follow-up CT chest and a free CXR, with a pause for CXR PAUS.

On 5/2/2026, the note reiterated the 12/7/2025 left mastectomy (lt mrm) and the July 2025 mdt plan of Zoledek, PORT, and new metastatic work-up. It also recorded the initiation of arimidex on 8/1/2026 and the administration of ARAMIDEX CALCIUM 1*1*90, ONE ALPHA ZOLADEX 10.8 SC SM. The breast lesion was again noted as likely fibroadenoma on RT imaging, with a BRIAD 3 for follow-up CT chest and a free CXR, and a pause for CXR PAUS.

On 30/04/2026, the oncology team noted that arimidex was given today and that the patient would receive the next two months of zoladex at the pharmacy.

On 28/07/2026, the internal medicine note documented the initiation of FEMARA plus supportive therapy for three months, along with ZOLADEX 20/10 clinic visits and ZOLADEX for thyroid goitre, with neck ultrasound and TFTs planned. The patient is currently receiving FEMARA and ZOLADEX as per the outlined schedule.

تحريرا في ,

التاريخ :	21-09-2026	**الوقت :**	

الطبيب المعالج : نهي سعيد علي محمد

يعتمد ,

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنت المصرية هي صحة مصر

MDT FORM 4/6/2025 G

Name	[illegible]	MRN	[illegible]
		ID	٣٠١٢٠٨٤٦٣٤١
Age	37 yrs	Phone	٠١٢٢٥٥٦٧٧٦٨
Menopausal status	Pre. Menopausal	Marital status	married
NRS SCORE		Family history	-ve
BMI		REASON OF MDT	[illegible]
CO morbidity	-ve		[illegible]

Clinical Data

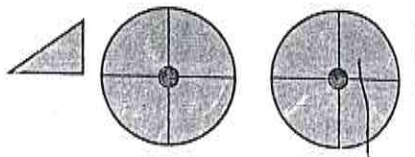
BX: IDC, G-II

ER:	PR:	HER 2:	Ki67:
8/8	6/8	0	18%

Sonomammography: Post NACT Bilat.
Mammo-sonography [5/5/2025]:
- Partial Response in Lt. Breast, Lt. axilla.
- Rt. Breast: BIRADS 3.

Mets Workup:
~~Recent~~
- CT CAP [10/2024]: Free.
- Bone scan [9/2024]: Free.

R L



TNM

Recommendations: Lt. Breast Ca. cT2 cN1 M0 (multi-centric)
- Dignosed in 9/2024 as locally Adv. Lt. Breast Ca., 2 clips inserted.
- Received NACT [4 AC → 4 Taxotere], last on 17/9/2025.
- Zoladex [For ovarian Function preservation].
- Post NACT Bilat. Mammo-sonography → done.
- MDT For surgery [Lt. Mammo-plasty inf. pedicle & acc. to target axillary LNs post NACT] was pre-planned.

MDT OUTCOME & PLAN

[illegible] c.co [illegible]
for Diffuse area of microCalcification patient is indicated for mastectomy
So for discussion of surgical decision with Dr Abdelsalam in out patient Clinic Mtaly

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

USTAVE OUSSY NATIONAL

بالتوفيق بعد العملية ☺

Patient Name	: الأستاذه / انتصار جمال محمد محمد	Request No.	: 13005389
Patient No.	: 28804120104524	Request Date	: 12/07/2025
Path No.	: 2247/25	Verification Date	: 19/07/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 37 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data:
Left breast cancer post NACT

Nature of the specimen:
Left simple mastectomy & Left Sentinel Lymph node biopsy & axillary lymph node dissection biopsy

Gross Examination:
- Procedure: Left simple mastectomy specimen measured 22x12x6 cm with covering skin ellipse measured 19x10cm .
- Tumor extent : A greyish white granular tumour bed site
- Size: 2.5x2x1 cm
- Skin involvement: Nipple & areola were unremarkable grossly
- Distance from margins: 0.2 cm away from deep margin
- Lymph nodes number and size: Fibrofatty tissue piece measured 4 x4 x1 cm enclosing 3 SLNs up to 1.5 x0.5 cm .
- Frozen section: Positive SLNs (2/3)

Microscopic examination:
- Histological type: Invasive duct carcinoma
(actual tumour size 2.5x2 cm) , (cellularity 60%)
- Grade (differentiation+ nuclear pleomorphism + mitosis):
Grade 2 (3+2+1)
- Angioinvasion: Positive
- Perineural invasion : Positive
- Ductal carcinoma in situ: Focal (10%), low grade (solid , cribriform patterns)
- Skin involvement: Free nipple and areola
- Lymph node involvement:

Verified by :
Dr.Heba Hasan

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

باثولجي بعد العمليه ①

Patient Name	: الأستاذه / انتصار جمال محمد محمد	Request No.	: 13005389
Patient No.	: 28804120104524	Request Date	: 12/07/2025
Path No.	: 2247/25	Verification Date	: 19/07/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 37 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

-Positive SLNs (2/3)
-Positive ALND (6/16) with extracapsular extension
-Total nodal count : Positive (8/19) with extracapsular extension
• Resection Margins: Free inked deep margin
• Histologic response to therapy: Pathologic partial response

Diagnosis:
Left Simple Mastectomy & Left SLN & Left axillary lymph node dissection (post NACT):
Invasive duct carcinoma grade 2, cellularity 60 %, actual tumor size 2.5 cm
• DCIS, low grade (solid, cribriform patterns) (10%)
• Free margins
• Free nipple and areola
• Positive LNs (8/19) with extracapsular extension
• Pathologic partial response (PPR)
• Stage : ypT2 , ypN2a
• RCB : 4.491 , RCB class III

Signature

Dr. Walaa Ibrahim Ghoneim
Dr.Heba Hassan Adly

Verified by :
Dr.Heba Hasan

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
الهيئة العامة للمراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Patient Name: Entesar Gamal Mohamed Mohamed Atteya	Date : 31/10/2024
Exam : C.T CHEST	Age : 36 Year(s)
Medical ID: 3651/2024	Pat. ID: 93789

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST , ABDOMEN AND PELVIS : -

I-Chest Findings:

- Left breast LOQ ill defined soft tissue mass lesion 3x2.4 cm with ipsilateral axillary lymphadenopathies, the largest about 2x1.1.8 cm. Left breast skin thickening.
- Right breast retroareolar soft tissue nodular density.
- Right upper lung lobe pleural based/subpleural pulmonary nodule 9x6 mm is seen.
- Otherwise, Clear both lung fields
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. *Minimal* pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.
- Enlarged left thyroid lobe and isthmus showing hypodense nodules and retrosternal extension for US correlation.

II-ABDOMEN AND PELVIS FINDINGS:

- The liver is seen mildly enlarged with diffuse homogenous low CT attenuation. No focal lesions on non enhanced CT basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Bulky spleen with no focal lesions seen.
- Normal size and parenchymal thickness of both kidneys with no back-pressure changes.
- Normal appearance of the pancreas and both supra renals.
- No abdominal lymph node gross enlargement seen.
- Bulky uterus with IUD seen in situ.
- No ascites or abdominal collections noted.

Opinion:

- **Left breast mass lesion with ipsilateral enlarged lymph nodes , right retro-areolar nodular density , for SM assessment.**
- **Minimal pericardial effusion.**

CT Scan
[[DOCTYPE:CT_SCAN]]

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Radiology Department

وزارة الصحة والسكان
الهيئة العامة للمراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Patient Name: Entesar Gamal Mohamed Mohamed Atteya	Date : 31/10/2024
Exam : C.T CHEST	Age : 36 Year(s)
Medical ID: 3651/2024	Pat. ID: 93789

- Right upper lung lobe subpleural nodule likely sequel of old inflammatory process.
- Otherwise, No evidence pulmonary active inflammatory disease, metastatic lung or hepatic deposits.
- No current osseous deposits.

Dr. Walid Mohamed, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
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Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Entesar Gamal Mohamed Mohamed Atteya	Date : 05/05/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 37 Year(s)
Medical ID: 3651/2024	Pat. ID: 93789

DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Indication:

Post NACT status assessment of a known left breast malignant mass compared to attached report dated 21/08/2024 with no available images for optimal comparison.

Report:

- ACR B: scattered fibro-glandular parenchyma of both breasts
- Left breast:
  - Left breast UOQ partially obscured residual dense architectural distortion. Two clips at UOQ and UIQ locations
  - Left breast UOQ pleomorphic segmental microcalcificaitons extending over an area of 76 X 93 mm in CC view.
  - Mild left breast skin thickening.
  - Ultrasound shows 1 o'clock irregular hypoechoic mass measures 11 x 18 mm.
  - The UIQ shows residual area of distortion on US.
  - Regressive course of the previously reported left axillary lymphadenopathy being reactionary in current study.
- Right breast:
  - Right breast inner domain deeply seated medium density rather defined lesion measuring 9 X 7 mm.
  - Complementary ultrasound showed no right breast 3 o'clock zone C epi-muscular area of altered parenchymal echogenicity measuring 9 X 7 mm.
  - Normal skin thickness. and nipple-areola complex with average caliber of the retro-areolar central ducts.
  - Nonspecific axillary LN.

Opinion:

- Post NACT status of left breast showing partial response of the previously noted malignant mass ipsilateral axillary lymphadenopathy (BIRADS 6).
- Left breast UOQ area of suspicious microcalcifications BIRADS-4.
- Right breast area of focal asymmetry with underlying area of altered parenchymal echogenicity mounting to mass formation (BIRADS 3) for histopathological assessment.

Dr. Mohamed Mohsen, M.D.

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient Name: Entesar Gamal Mohamed *Date: 29/9/2024*

Referring physician: Prof.

^{99m}Tc-MDP BONE SCAN

Desired information: *skeletal surveying to detect multiple lesions.*

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:

- No definite active hot spot that could account for osseous metastatic lesions .
- Spondylodegenerative changes are seen at the spine ,.
- Mild osteoarthritic changes are seen at both shoulders , both knees .
- Both kidneys are normally seen.

Conclusion:

The surveyed skeleton is free of any osseous metastatic lesions

Thanks for the referral

Nuclear Medicine Unit

DR . Sherin Mahdy, MD

DR/ Mohamed Emam

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Patient Name: Entesar Gamal Mohamed Mohamed Atteya	Date : 06/08/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 37 Year(s)
Medical ID: 3651/2024	Pat. ID: 93789

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST , ABDOMEN AND PELVIS : -

I-Chest Findings: compared to last study dated 31/10/2024

- Left MRM with ill defined mass measuring 3.6 x1.6 cm with overlying skin thickening.
- Right breast retroareolar soft tissue nodular density.

Mild progression of

- multiple bilateral pulmonary nodule, the largest seen at left lower lobe measuring 4mm
- few mediastinal lymph nodes, the largest at pretracheal group 9x4mm.

- Otherwise, Clear both lung fields
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- Resolution of pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.
- Enlarged left thyroid lobe and isthmus showing hypodense nodules and retrosternal extension for US correlation.

II-ABDOMEN AND PELVIS FINDINGS:

- The liver is seen midldly enlarged with diffuse homogenous low CT attenuation. No focal lesions on non enhanced CT basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Bulky spleen with no focal lesions seen.
- Normal size and parenchymal thickness of both kidneys with no back-pressure changes.
- Normal appearance of the pancreas and both supra renals.
- No abdominal lymph node gross enlargement seen.
- Bulky uterus with IUD seen in situ.
- No ascites or abdominal collections noted.

Patient Name: Entesar Gamal Mohamed Mohamed Atteya	Date : 06/08/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 37 Year(s)
Medical ID: 3651/2024	Pat. ID: 93789

- Examined skeleton showas small osteolytic lesion 2mm at left iliac bone .

Opinion:

- **Mild progression of pulmonary nodule and few mediastinal lymph nodes.**
- **small osteolytic lesion mm at left iliac bone**
- **Rather stationary rest of study**

Dr. Reham Khedr, M.D.

Signature Reeham.Khader
11/08/2025 1:30 AM





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